MS-BLS-29
Basic and Advanced Cardiac Life Support (BLS & ACLS)

MODULE 00

Course Handbook and Study Guide

How this course is structured, what each module covers, how you will be assessed, and what this course can and cannot certify.

Learning hours	Read before you begin
Who this module is for	All learners enrolled in MS-BLS-29
Assessment	Not assessed — orientation document
Regulatory basis	AHA 2025 Guidelines for CPR and Emergency Cardiovascular Care; NABH 6th Edition

This module is one part of a certified course. Content is current as at August 2026. Fire safety requirements vary between States and are set for your building by its approved drawings and Fire NOC conditions; verify against those and against your hospital's own fire plan before relying on this material for a compliance decision.

MS-BLS-29 — Course Handbook

Cardiac arrest is the only emergency in which the actions of the nearest person in the first three minutes matter more than everything done by everyone else afterwards. This course is built around that fact. It is written against the 2025 American Heart Association Guidelines for CPR and Emergency Cardiovascular Care, released in October 2025 — a comprehensive revision of the 2020 guidelines.

What this course can and cannot certify

This is a knowledge course. It builds understanding of resuscitation science, the algorithms, and the hospital system around them, and it produces a MedSkills certificate of completion. It is NOT a substitute for a hands-on BLS or ACLS provider course with manikin-based skills assessment, and it does not confer an AHA, ERC or equivalent provider card. Compressions, bag-mask ventilation and defibrillator operation are psychomotor skills that must be assessed physically at an authorised training centre. Use this course to prepare for and reinforce that training — not to replace it.

How the course is organised

Eleven modules, issued as separate documents so departments can assign what each role needs. Modules 1 to 5 are the BLS core and are for every clinical staff member. Modules 6 to 11 carry the advanced life support, peri-arrest, post-arrest and systems content for those who respond to or lead codes.

Module	Title	Hours	Core audience
01	Cardiac Arrest and the Chain of Survival	1.0	All clinical staff
02	Recognising Deterioration, Arrest and Calling for Help	1.0	All clinical staff
03	High-Quality Chest Compressions	1.0	All clinical staff
04	Airway, Ventilation and Choking	1.0	All clinical staff
05	Defibrillation and the AED	1.0	All clinical staff
06	The Adult Cardiac Arrest Algorithm and Drugs	1.25	Code responders, doctors, ICU and ED nurses
07	Advanced Airway Management and Monitoring	1.25	Anaesthesia, ICU, ED, code team
08	Peri-Arrest Rhythms: Bradycardia and Tachycardia	1.25	Doctors, ICU and ED nurses, code team
09	Post-Resuscitation Care, Prognostication and Stopping	1.25	ICU, ED, code team, physicians
10	Resuscitation in Special Circumstances	1.25	Obstetrics, anaesthesia, ED, ICU, dialysis, theatre
11	Team Performance, Debriefing and the Programme	1.25	Code team, resuscitation officers, quality

Total course duration: 13 hours

Comprising approximately 9 hours of self-paced module study, 2.5 hours of applied practice and scenario work, and 1.5 hours of assessment. This is in addition to the hands-on provider course and skills assessment required for BLS or ACLS certification.

A note on the 2025 Guidelines

The AHA released the 2025 Guidelines for CPR and ECC on 22 October 2025. If you trained on the 2020 guidelines, most of what you know remains correct — you are updating, not relearning. The compression rate of 100 to 120 per minute, the 30:2 ratio for single-rescuer adult CPR, the reversible causes and the overall shape of the algorithms all still hold.

What changed matters, though. There is now a single unified Chain of Survival covering adults and children, in-hospital and out-of-hospital. The terminology has been revised: ‘rescue breaths’ is no longer used, ‘breaths’ now refers to assisted breathing, and ‘ventilation’ is reserved for mechanical respiratory support. Opioid antagonists have entered the BLS algorithm for healthcare professionals, there is a new choking algorithm, and in shockable rhythms epinephrine is now positioned after initial defibrillation attempts have failed rather than alongside them. Vasopressin is no longer recommended as a substitute for epinephrine. Each module notes where the 2025 position differs from 2020.

A note on learning hours

NABH does not prescribe a fixed number of training hours for any individual topic. What the 6th Edition requires is that the organisation identifies training needs, provides training including resuscitation training for relevant staff, and maintains records of the date, duration and effectiveness of that training. The hours above are MedSkills’ recommended durations, recorded automatically against each learner in the LMS. Your hospital’s own resuscitation policy will set the certification and recertification interval, and that is binding.

Assessment and certification

- Each module carries a knowledge check written in the style of the final assessment.
- The final assessment covers all eleven modules. The pass mark and permitted attempts are set by your institution.
- On passing, a verifiable MedSkills certificate of knowledge completion is issued, with date and duration written to your training record.
- This certificate records knowledge, not psychomotor skill. Your provider card must come from a hands-on course at a recognised training centre.

What you should be able to do at the end

1. Recognise cardiac arrest within seconds and start compressions without waiting for permission or for the team.
2. Describe high-quality CPR precisely — rate, depth, recoil, interruptions — and explain why each parameter matters.
3. Operate an AED and describe safe manual defibrillation, including pad placement and minimising the peri-shock pause.
4. Follow the adult cardiac arrest algorithm and state the drugs, doses and timing.

5. Work through the reversible causes systematically rather than reciting them.
6. Assess a peri-arrest bradycardia or tachycardia for instability and act accordingly.
7. Deliver the immediate post-ROSC bundle and explain the harm of hyperoxia and hyperventilation.
8. Modify resuscitation appropriately in pregnancy, hyperkalaemia, hypothermia, anaphylaxis and poisoning.
9. Function within a code team using closed-loop communication, and speak up when something is wrong.

How to use these documents

- Read the modules in order; each builds on the previous one.
- Attempt every knowledge check before reading the answers.
- Print the algorithm figures and post them beside the resuscitation trolley.
- Bring this material to your hands-on provider course; the psychomotor training is far more effective when the reasoning is already in place.
- Practise. Skills decay within months. Brief, frequent practice beats a long course every two years.

> **Currency and local protocol**
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> This content is current as at August 2026 and is based on the AHA 2025 Guidelines. Resuscitation guidance is revised periodically and regional councils — including the European Resuscitation Council and the Indian Resuscitation Council — publish their own guidance which may differ in detail. Drug doses, energy settings and targets must always be checked against your own hospital's resuscitation policy and your own device's recommended settings. Where your hospital protocol differs from this material, follow the hospital protocol and raise the discrepancy with your resuscitation committee.

Figures in this course

All diagrams are original works created for MedSkills. No third-party stock imagery is used, which means the algorithms, doses and parameters shown are drawn to match the guidelines they cite rather than approximating them. They may be reproduced within the licensed institution for training and display purposes.